06 — Implementation Plan

Framework: **Model for Improvement** (Langley et al.) with sequential PDSA cycles, reported with **SQUIRE 2.0**.

Three questions

1. **What are we trying to accomplish?** See the SMART aim in 00.
2. **How will we know a change is an improvement?** See the measures in 07.
3. **What changes can we make?** See the driver diagram below.

Driver diagram

Aim	Primary driver	Change ideas
≥ 80 % of visits with an up-to-date screen; **≥ 90 %** of positive screens offered a referral	**Standard tool & workflow**	Adopt the adapted AHC-HRSN form · "Screen due?" flag at intake · Scoring key on the form
	Staff capability & buy-in	2-hr training + competency check · Screening champion · Huddle reminders + weekly run chart
	Patient trust & privacy	Intro script, opt-out · Private space for safety items · Patient-advisor review of wording
	Reliable response to needs	Verified resource directory · Warm handoff when possible · Resource card (EN/ES) · 30-day follow-up tracking

Phases (default 16 weeks; timeline not yet decided)

Phase	Weeks	Key activities	Exit criteria
0. Prepare	−4 to 0	Sponsor approval; QI/IRB determination; verify resources; patient-advisor review; train staff; print materials	Approvals done; 100 % screeners pass competency
1. Baseline	1–4	Measure current practice: chart audit of all adult visits (was *any* social need asked and documented? referral?)	≥ 4 weekly data points
2. PDSA 1	5–8	Champion screens every patient who is due, at intake (paper)	Screening ≥ 60 %; workflow time known
3. PDSA 2	9–12	Spread to all trained staff; add desire-for-help and referral-offer step	Screening ≥ 80 %; referral offer ≥ 90 %
4. PDSA 3	13–16	Add 14-/30-day follow-up and closed-loop tracking	Follow-up documented ≥ 50 %

Phase	Weeks	Key activities	Exit criteria
5. Sustain	17+	Add to onboarding; monthly audit; hand off to clinic owner	Sustained ≥ 80 % for 3 months

PDSA cycle plans

PDSA 1: Champion-led screening at intake

- **Plan:** A single trained champion screens every adult who is due, at intake with the paper form. *Prediction:* screening rises from about 25 % to at least 60 %, with fewer than 8 minutes added per visit.
- **Do:** Run for 4 weeks. Log visit time, declines, and barriers (no private space, interpreter needed).
- **Study:** Review the weekly run chart, decline rate, and time per screen. Debrief the champion.
- **Act:** Adopt, adapt (for example, move screening to the waiting period), or abandon.

PDSA 2: Spread + referral offer

- **Plan:** All trained staff screen. Each positive domain gets the desire-for-help question and a referral offer. *Prediction:* screening ≥ 80 % and referral offer ≥ 90 %.
- **Study:** Compare screening rate by screener role (fidelity) and the referral funnel.

PDSA 3: Closed-loop follow-up

- **Plan:** The referral owner follows up at 14 ± 7 days and again by 30 days. *Prediction:* ≥ 50 % of accepted referrals have a documented outcome.
- **Study:** Track the unable-to-reach rate and which contact methods work.

Use templates/pdsa-worksheet.md for each cycle.

Stakeholder and communication plan

Who	What	When
Clinic staff	Run chart on the wall/van + huddle	Weekly
Clinic sponsor	1-page update	Every 4 weeks
Faculty advisor	Progress + data	Per program schedule
Community partners (WFCM, shelters, CSP)	Heads-up on expected referral volume; feedback	Before go-live + monthly
Patients	Resource card; “we ask everyone” signage	Ongoing

Barriers and mitigations

Barrier	Mitigation
Limited private space in the van	Use an outdoor/curtained area for safety items, or defer and record “deferred”
Interpreters	Phone interpreter line; translated forms
Volunteer/staff turnover	Job aid; champion; onboarding module

Barrier	Mitigation
Resources at capacity	Re-verify quarterly; track “not connected” reasons
Patients without phones	Collect a shelter/case-manager contact; follow up at the next stop; Lifeline referral

Sustainability

- Add the screening to the standard visit template.
- Assign an owner for monthly audit (10 random charts).
- Re-verify the resource directory quarterly.
- Phase 2 tool decision (09).